

Transforming Certification for Better Care

ABMS CONFERENCE 2026

Unionization for Trainees & Practicing
Physicians: Impact* on Trainees, Programs, and
Boards

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Disclosure

- I have nothing to disclose

Unions brought us weekends and more...

Minimum
wage

40 hour work
week

An end to
child labor

Workplace
safety
protections

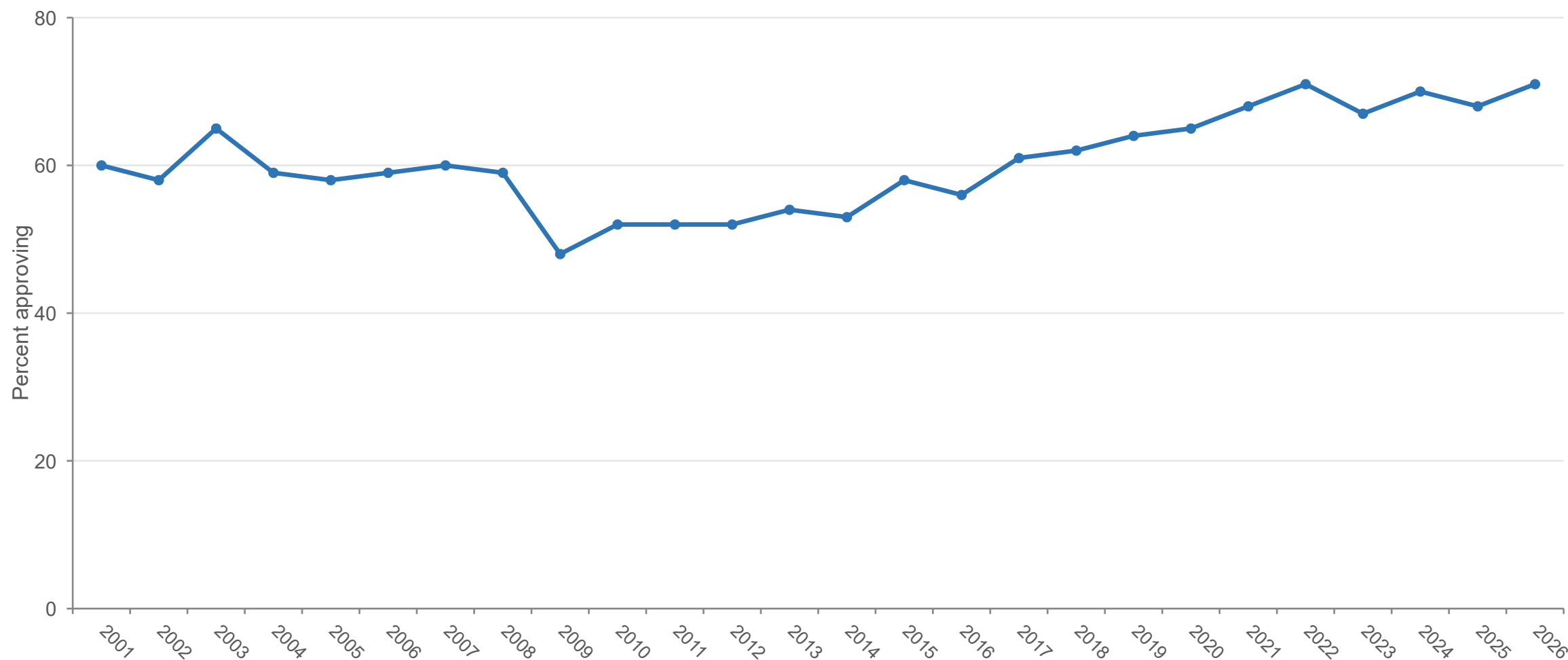
Paid vacation

Healthcare
benefits

Pensions

Public Approval of Labor Unions Is Steadily Increasing

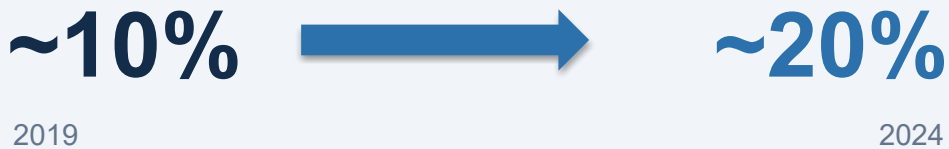
Gallup: "Do you approve or disapprove of labor unions?"



Resident & Fellow Unionization Is Rising Rapidly—California Is Far Ahead of the

Representation has approximately doubled nationally since 2019, while California now exceeds four in five residents represented by CIR.

UNITED STATES



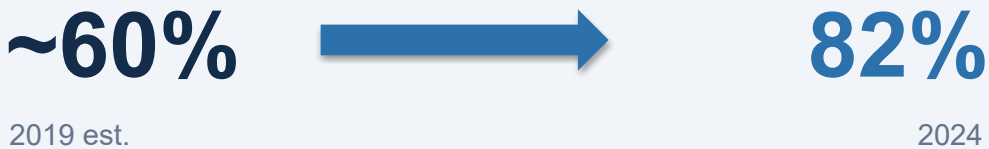
CIR representation roughly doubled

CIR membership grew from ~16,000 residents/fellows in 2019 to ~32,000 by May 2024—and >37,000 by early 2025.

7

NLRB housestaff union elections in 2023 vs. 4 in 2022

CALIFORNIA



More than 4 in 5 residents represented

California has the largest concentration of physician-in-training unions nationally;
CIR reported 82% representation after the 2024 Kaiser Permanente Northern California campaign.

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physician-in-training unions identified in California in the national JAMA analysis

TAKEAWAY

California is not simply following a national trend—it is operating at a substantially higher level of housestaff union penetration.

California Stories

Stanford: 2023

- 21% Wage Growth/3 years
- Fully Funded Ride Share for Fatigue Mitigation
- \$50,000 Annual Wellness Fund

Kaiser:2024

- 14.5% Wage Growth/3 years
- Housing, Meals, Conference Funds
- Annual \$40,000 patient-focused project fund

UC: Consolidated: 2026

- 18% Wage Growth/3 years
- ED Crowding
- Protective Gear
- Protections for immigrant patients
- Fertility Benefits

Residents are Inclined to Unionize; Early Career Practicing Physicians Are Not Far Behind.

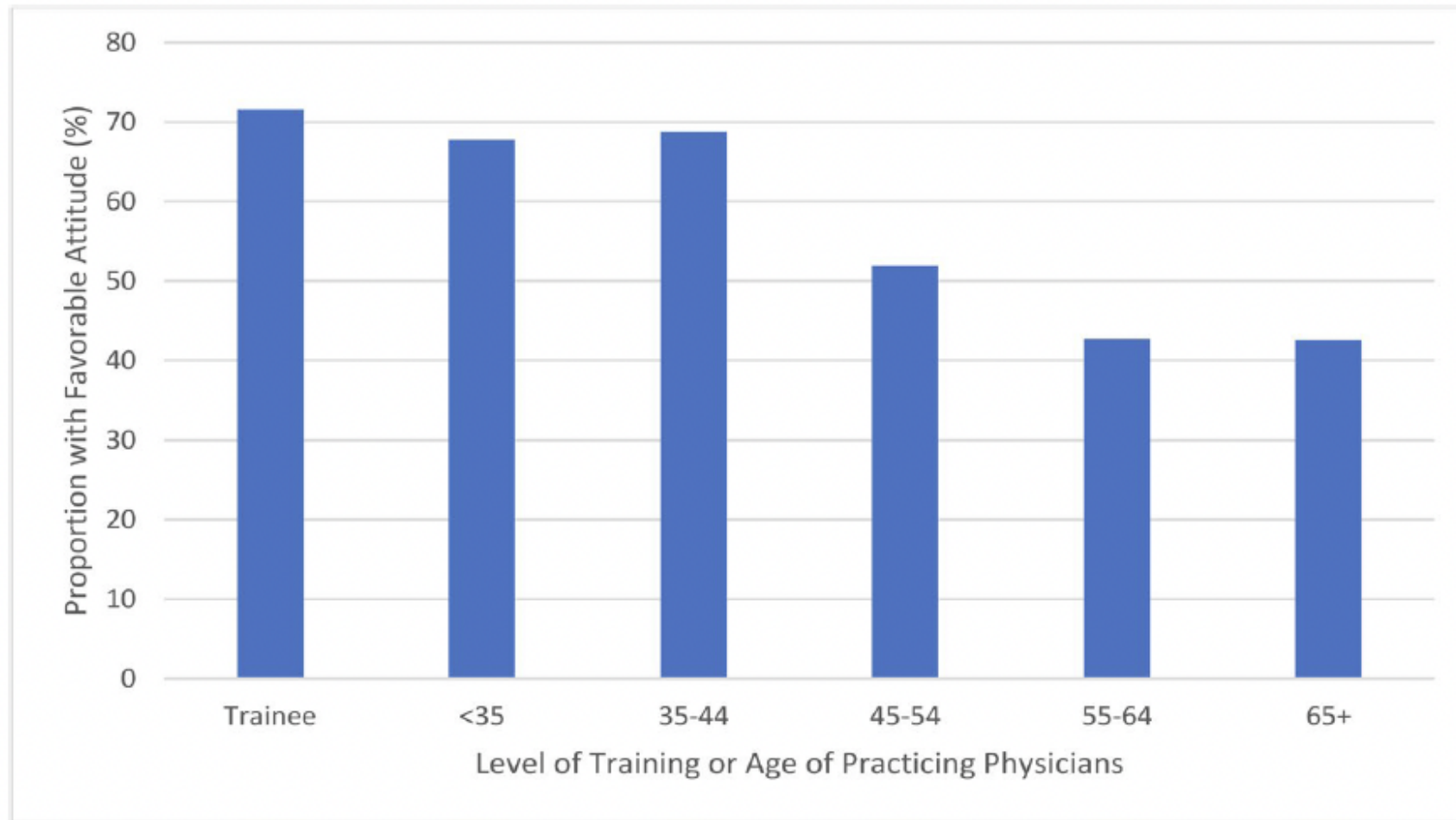
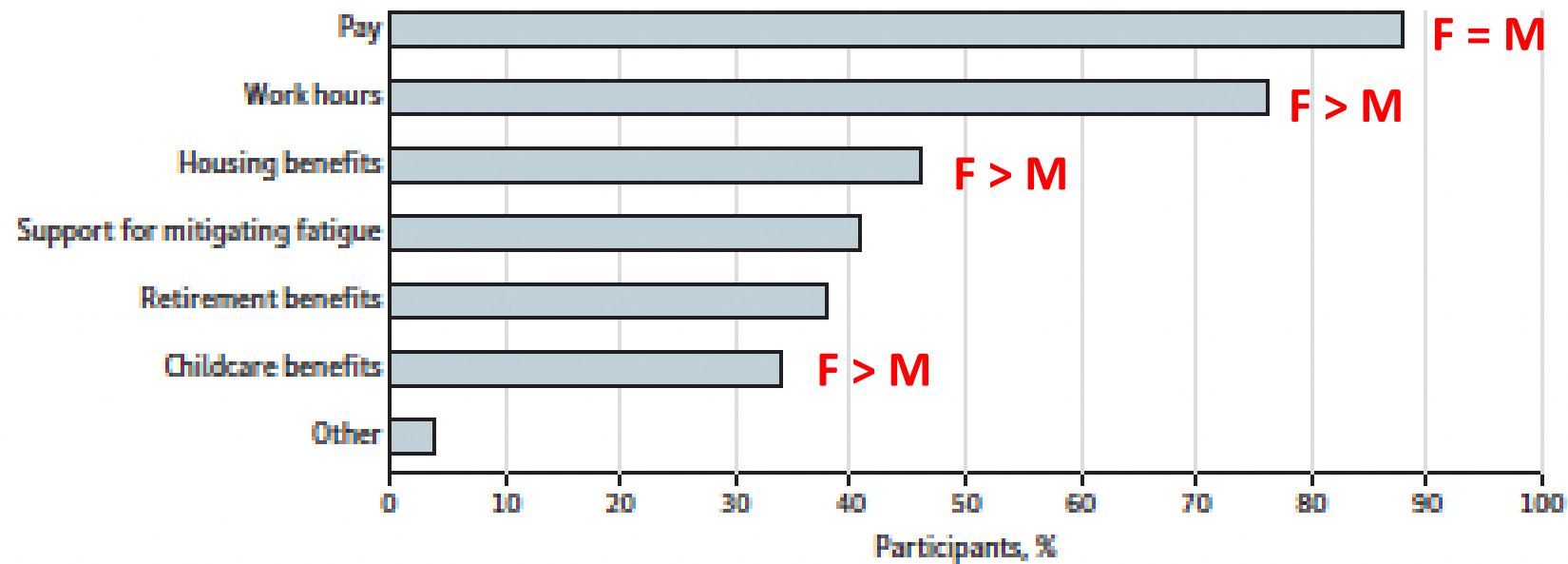


Figure 1 Unionization attitudes of responding trainees and practicing physicians by age in years, % with favorable or strongly favorable

Drivers of Unionization

Figure 2. Factors Reported by Resident Physicians as Important in Thinking About Unionization



Absence of Certain Benefits Increases the Likelihood of Voting to Unionize

Table 2. Association Between Lack of Specific Benefits and Likelihood of Voting for a Union

Benefits	OR (95% CI)	P value
Housing stipend	1.04 (0.61-1.77)	.88
Contribution to retirement fund	1.41 (0.91-2.18)	.12
Need-based stipends	3.41 (1.25-9.30)	.02
Paid maternity leave	1.69 (1.08-2.64)	.02
Paid paternity leave	1.77 (1.15-2.73)	.01
Childcare facilities	3.13 (1.38-7.09)	.01
Funding for childcare	1.80 (0.49-6.58)	.37
Coverage if your child is sick	2.10 (1.11-3.98)	.02
Flexibility to attend child's doctors' appointments	1.85 (1.10-3.11)	.02

Impact of Unionization on Residents/Fellows Work Life?

- Unionized programs were more likely to offer housing stipends and more generous vacation (4 weeks vs 2-3 weeks)
- No difference was seen between unionized and nonunionized residents in these areas:
 - burnout,
 - suicidality,
 - job satisfaction,
 - duty hour violations,
 - mistreatment,
 - salary, or
 - the educational environment

Unintended consequences of unionization for trainees

- Loss of department specific benefits
- Inability to utilize negotiated benefits (such as childbearing/childrearing) because of Board caps on leave before requiring extension of training
- Conflict between CBA and ACGME rules (ex moonlighting)
- Subjecting underperforming trainees to disciplinary action rather than educational interventions

Concerns of DIOs and Boards

- The adversarial nature of contract negotiations, which focus on institutional shortcomings and not on successful collaborations
 - Risk of Direct Dealing grievances if Programs “correct the record”
- CBA may not differentiate between specialties, which may limit program by program management of the working/learning environment, benefits, etc
- Arbitration may be required for dismissal of represented residents
 - California statute: . *Gov. Code § 3571.5(a)(2)*. Prohibits use of binding arbitration for academic or clinical matters; however:
 - Challenges to whether “professionalism” is academic or not, specifically around behaviors that employees might be disciplined (dress code, tardiness, unexcused absences)

Opportunities to Consider

- Programs
 - Plan strategically to give “Voice and Choice” trainees with departmental and institutional leaders
 - Empower trainees to develop problem solving skills through collaboration to address quality, safety, and learner experience issues
 - Develop and be transparent about performance management and due process
- Boards
 - Learn from union contracts what today’s physicians need to thrive throughout their careers
 - Engage new diplomats to review board standards for legacy practices that may not be needed in today’s environment